FICTIONAL - FOR DEMO

Riverside Community Hospital - Discharge Summary

Patient: Taylor Brooks (fictional), 46-year-old
Admission date: November 2, 2026
Discharge date: November 5, 2026
Attending: Dr. Alicia Moreno, Hospital Medicine

Diagnosis: Cellulitis of the left lower leg.

Hospital course: You came in with a red, warm, painful left lower leg and fever. Blood tests and an ultrasound showed an infection of the skin without a blood clot. You received IV antibiotics, the redness has started to shrink, and your fever is gone.

Medications:
Cephalexin 500 mg by mouth every 6 hours (4 times a day) for 7 days. Take it even if the leg looks better.

Diet:
Eat a normal, balanced diet with enough protein to help your skin heal.
Drink plenty of water.

Activity:
Keep your left leg raised on pillows above heart level when you sit or lie down.
Walk short distances every few hours. Avoid standing for long periods.

Wound care:
Wash the area gently with soap and water once a day and pat it dry.
Moisturize the dry skin around the area, but not on any open sore.
Wear clean, loose socks and shoes that do not rub.

Monitoring:
Use a pen to mark the edge of the redness today. Check the mark every morning and evening for 7 days to see if the redness spreads.
Check your temperature every evening for 7 days.

Follow-up:
See Dr. Alicia Moreno, primary care on November 8, 2026 (3 days after discharge) for a skin check and to review the redness.
See Wound care clinic on November 19, 2026 (14 days after discharge) for a final skin check.

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Call 911 or return to the ER if:

- trouble breathing
- fainting or confusion
- a fast heartbeat with shaking chills
- skin turning purple or black, or blisters filling with dark fluid
- severe pain far worse than the way the leg looks

Call the clinic at 555-0111 if:

- redness spreading past the line you drew
- a fever of 101 F (38.3 C) or higher
- pus or drainage from the area

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